

**جوستاف روسي انترناسيونال - مصر (EPE) \* طلب علاج أول مرة / متردد**

إِسْمَ الْمَرِيضِ رُبَاعِيًّا : هبه السيد على محمود

### المحافضة : الجيزه

## الحى / المدينة : يولاق الذكرور

**العنوان : بولاق الدكرور**

رقم البطاقة : 28606302300861      رقم التليفون : 01063910027

**تاريخ تسجيل الإستمارة : 20-09-2026**

**العمر(تاريخ الميلاد) : 30-06-1986 المهنة : ربه منزل**

## الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي  
التوقيع: (\_\_\_\_\_)

المقر بما فيه  
التوقيع: (-----)

## التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

## نتائج الأبحاث : (مرفق)

### التشخيص النهائي : سرطان بالثدي (مبادرة)

**خطة العلاج :** علاج مبادرة سرطان الثدي علاج تدعيمى وتلطيفى [6579] **قسم العلاج :** الاورام

**التعليق على التشخيص : -----**

## تقرير اللجنة الثلاثية المتخصصة

**الاسم: د/محمد کیلانی**

**الاسم: د/ندی حسن**

**الاسم:** د/نرمين رمضان

## الوظيفة:

## الوظيفة:

## الوظيفة:

## التوقع:

## التوقيع:

## التوقيع:

تعتمد صحة البيانات المرفقة  
رئيس اللجنة ومدير  
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.**

## أعضاء اللجنة

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

## قرار اللجنة

### التشخيص النهائي :

### خطـة العـلاج :

### المستشفى :

### جهة العلاج : --

### ملاحظات : ---

**قسم :**

### نفقات :

## ختم

## شعار الجمهورية

## مدير عام

## مديرية الشؤون الصحية

## رئیس

## المجلس الطبي المتخصص

|  |
|--|
|  |
|--|

|                                                                        |                                                                                     |
|------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> هبه السيد على محمود<br><b>National ID:</b> 28606302300861 |  |
|------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بأن نوضح فيما يلي تقريراً عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد أنه |
|----------------------------------------|

|                                                                                      |
|--------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment |
|--------------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |



دms  
الينو



عاجل

06/24/2026

## MDT FORM

|                                 |                           |
|---------------------------------|---------------------------|
| <b>Name:</b> هبة السيد على محمد | <b>MRN:</b> 10851         |
| <b>Age:</b> 40 Y                | <b>ID:</b> 28606302300861 |
| <b>Menopaus Status:</b>         | <b>Phone:</b> 01062080399 |
| <b>NRS SCORE:</b>               | <b>Marital Status:</b>    |
| <b>BMI:</b>                     | <b>Family History:</b>    |
| <b>CO Morbidity:</b>            | <b>Reason Of MDT:</b> new |

## Clinical Data:

**BX:** idc g2 .. with focal lobular component  
**ER** 8 **PR** 6 **HER** +2 nc **KI67** 40

## Sonomammography:

LEFT BREAST:

Low energy (Non contrast) images:

Picture of diffuse breast edema in the form of thickened skin and coarsened trabeculae.

Global asymmetry affecting central breast anterior & middle thirds, reaching the nipple and showing surrounding architectural distortion and UOQ regional coarse heterogeneous micro-calcifications seen within measuring 2.5 x 2.5 x 3 cm.

Two UOQ posterior third intra-mammary lymph nodes.

Recombined (Post contrast) images show heterogeneous non-mass enhancement of high conspicuity occupying most of the breast (relatively sparing the posterior third) and reaching the nipple measuring 7.7 x 3.9 x 4.5 cm.

Complementary ultrasound:

Diffuse breast edema pattern in the form of thickened skin reaching 8 mm.

Central breast malignant mass lesion occupying most of the breast and reaching the nipple.

Two UOQ intra-mammary lymph nodes, one is pathological at 1 o'clock measuring 13 x 6 mm with eccentric hilum and cortical thickness reaching 6 mm, while the smaller one is seen at 2 o'clock showing non-specific characters with cortical

## Mets Workup:

R

L



TNM





Ministry of Health and Prevention  
Kingdom of Saudi Arabia

وزارة الصحة والوقاية  
المملكة العربية السعودية



## MDT form

A-Patient ID:

1- Name: محمد بن علي محمد

Sex:

2- Medical number:

Female

03163214

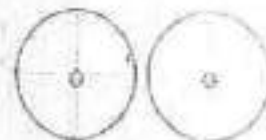
Old Med:

Date:

3/9/2023

3-ID number: 28501021191423

Patient Age: 49 Y



|                |                |                   |                   |               |               |               |
|----------------|----------------|-------------------|-------------------|---------------|---------------|---------------|
| Family history | Family history | Phone             | 01294457349       | WH            | REASON OF MDT | Reason of MDT |
| marital status | Marital status | WH Score          | WH Score          | GROUP         | Date of MDT   | Date of MDT   |
| Co-morbidity   | Co-morbidity   | Nonopposed status | Nonopposed status | Initial STAGE | Final STAGE   | 11/05/2024    |

## Clinical data

Breast Imaging:

Histo Workup:  
MBC to bone

|           |         |         |           |         |         |
|-----------|---------|---------|-----------|---------|---------|
| BR        | ER      | PR      | Her 2 Neu | SISH    | Ki67    |
| IOC       | 0/0     | 3/3     | 3         |         | 20      |
| 10/100000 | 0/00000 | 0/00000 | 0/00000   | 0/00000 | 0/00000 |
| Y         |         | N       |           | M       | 1 Bone  |

Sonomammography

## PATIENT HISTORY

started 27 cycles taxol+ 9 cycles herceptin last 17/4/2024-->clinical progression  
ct: D12 collapse SD  
BS: mild progression  
pt received RTH to D12  
New Bone scan showed PD  
shifted to navelbine but pt had poor compliance to navelbine.

## MDT OUTCOME &amp; PLAN

Shift to Gemzar (full dose) / Tykerb (with dose reduction).

## SIGNATURE

|                  |  |           |  |
|------------------|--|-----------|--|
| MEDICAL ONCOLOGY |  | RADIOLOGY |  |
| SURGERY          |  | PATHOLOGY |  |
| Psychiatric      |  | Nutrition |  |

**Patient's Name:** neama sobhy shalabi mohamed  
**Date:** 2026-04-02  
**Referred by:** Prof.Dr. ct cap iv  
**Hospital:** Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:**

**I- CHEST:**

- Known case of breast cancer
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

**II- ABDOMEN AND PELVIS:**

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulæ or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple scattered sclerotic lesions seen at the vertebral bodies, bilateral iliac bones and left clavicle

**Opinion:**

- ❖ Known case of breast cancer
- ❖ Multiple bony metastatic lesions as described

*Dr.Asser Ahmed*

*Dr.Asser Ahmed*

*Dr.Asser Ahmed*

Patient's Name: neama sobhy shalabi mohamed  
Date: 2026-04-02  
Referred by: Prof.Dr. ct cap iv  
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND  
PELVIS REVEALED:**

**I- CHEST:**

- Known case of breast cancer
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

**II- ABDOMEN AND PELVIS:**

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple scattered sclerotic lesions seen at the vertebral bodies, bilateral iliac bones and left clavicle

**Opinion:**

- ❖ Known case of breast cancer
- ❖ Multiple bony metastatic lesions as described

*Dr.Asser Ahmed*

*Dr.Asser Ahmed*

*Dr.Asser Ahmed*



Presidential Initiative for Women's Health  
Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - مستشفى الباطن  
مستشفيات جامعة القاهرة

**PATHOLOGY REPORT**

Name: أسماء محمد شبيب

Patient's I.D.: 28501031401423

Age: 38 Y.

Clinical data: Left breast locally advanced mass with diffuse skin thickening.  
Nature of the specimen: U.S. Guided Tru Cut Biopsy.

Pathology Number: 351 /2023

Specimen Received Date: 30 / 08 /2023

Date Report Issued: 7 / 09 /2023

**GROSS EXAMINATION:** Tissue core biopsy, totally submitted.

**MICROSCOPIC EXAMINATION:** Serial sections examined revealed cores of malignant tumor formed of groups, cords and cribriform structures of moderately anaplastic ductal epithelial cells. Surrounding desmoplastic stroma with ~5% TILs was seen. Tiny skin covered fragment was seen showing dermal proliferated capillaries with dense widely crushed inflammatory cellular infiltrate. Area of fat necrosis was also seen.

**IMMUNOHISTOCHEMISTRY PROCEDURE:** Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2), CONFIRM™ anti-Her-2/neu (1B5) Primary Antibody), and CONFIRM anti-Ki67 (30-2) Primary Antibody, using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

**IMMUNOHISTOCHEMISTRY RESULTS:**

Examination of section treated for estrogen receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.  
Examination of section treated for progesterone receptors revealed strong positive nuclear staining in 70% of tumor cells by chromogen.  
Examination of section treated for Her-2/neu revealed strong complete membrane staining in most of tumor cells by chromogen (score 3+).  
Examination of section treated for Ki67 revealed positive nuclear stain in average of 70% of tumor cells.

**DIAGNOSIS:** Left Breast Mass, U.S. Guided Tru Cut Biopsy:

- Invasive Duct Carcinoma, NST, Grade II.
- **Positive** For Estrogen Receptors, Score 8/8.
- **Positive** For Progesterone Receptors, Score 8/8.
- **Positive** For Her-2/neu Expression (Score 3+).
- **70%** Ki-67 Proliferative index.

**COMMENT:** All controls show appropriate reactivity.

DR. SARA ABOU-FANDOUD  
DR. PASSANT ESSAM SHIBEL



Patient Name: Ne'ma Sobhy Shalabi Mohamed

Exam: CT CAP+C (CT Chest Abd. Pelvis)

Medical ID:

Date: 13/10/2025

Age: 40 Year(s)

Pat. ID: 42376

## POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

- As Compared to the previous study dated 27/05/2025.

### I Chest:

#### • Still noted rather unchanged:

- Left breast parenchymal distortion with diffuse skin thickening.
- Few subcentimetric prominent left axillary lymph nodes largest measuring 5mm in short axis.
- No sizable worrisome pulmonary masses, areas of consolidation or cavitary lesions.
- No sizable pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

### II Abdomen & pelvis:

- The liver is still seen mildly enlarged with diffuse fatty infiltration with Stable size of the previously noted hepatic dome (Segment VIII) small hypodense focal lesion measures about 6 mm with peripheral nodular enhancement? hemangioma.
- No biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The scanned skeleton shows:
  - Stable few lytic osseous lesions involving both iliac bones and vertebrae, with still noted D12 vertebral collapse & L5 central wedging.

### Opinion:

- Follow-up status showing rather stationary course since the last study.

Signature Prof. Ali Remah  
14/10/2025 05:20.





Exam Information:

|                                       |                                       |
|---------------------------------------|---------------------------------------|
| <u>Patient Name:</u> Neama Sobhy      | <u>Date of birth:</u> 30/10/1985      |
| <u>Arabic Name:</u> نعمة صبحي         | <u>Patient Gender:</u> F              |
| <u>Patient RIS ID:</u> 368863         | <u>Exam Date:</u> 05/05/2025          |
| <u>Patient HIS ID:</u> 10110225007430 |                                       |
| <u>Written by:</u><br>Dr. Hoda Pathy  | <u>Approved by:</u><br>Dr. Hoda Pathy |

<sup>99m</sup>Tc MDP Bone Scan Report (30/4/2025)

\* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post IV Injection of 20mCi of <sup>99m</sup>Tc MDP  
Revealed:

Unfortunately, Previous bone scan images are not available. Limited comparison done with report dated 24-9-2024 revealed:

- Currently seen low grade non-uniform tracer uptake is seen involving DV12/LV1 ?regressing metastatic deposits, for CT cors correlation.
- The rest of the skeleton is currently scintigraphically free.
- Degenerative changes are noted over the rest of the spine, more marked over the upper cervical vertebrae for CT correlation.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- Follow up bone scan shows ?regressing bone deposits at DV12-LV1 with equivocal changes at upper cervical vertebrae for CT correlation and serial bone scan images.



Cairo University  
National Cancer Institute  
Department of Diagnostic and  
Interventional Radiology



جامعة القاهرة  
المعهد القومي للأورام  
قسم الأشعة التشخيصية والتداخلية

Much Oblived

Mahmoud Akram

Dr. Ahmed Zaher, MD



|                                           |                  |
|-------------------------------------------|------------------|
| Patient Name: No'ma Sobhy Shalabi Mohamed | Date: 27/02/2025 |
| Exony: ultrasound ABDOMEN & PELVIS        | Age: 10 Year(s)  |
| Medical ID: 3492/2023                     | Pat. ID: 42376   |

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION  
REVEALED:**

**Liver: :**

- Average in with homogenous echopattern
- No intra hepatic biliary radicle dilatation or focal lesions.
- Portal vein is patent

**Gall Bladder: -**

- Average distention with normal wall thickness with no stones.
- CBD is not dilated.

**Spleen: -**

- Average in size displaying homogenous echopattern, no focal lesions.

**Kidneys: -**

- Are of normal size, well differentiated & no gross stones or back pressure changes or space occupying lesion could be seen.

**Para-aortic and pancreatic area: are concealed by gases.**

**No ascites or free fluid or collection seen.**

**Urinary Bladder: -**

- Minimally distended no stones or focal lesion seen
- **Uterus :** Average size with homogenous echopattern, with no focal lesions.
- No ovarian cysts or adnexal lesions could be detected.
- No abnormal bowel dilatation

**Opinion:**

- Apart from colonic distension, Unremarkable study

Signature Dr. Sherien Ali El Sharkawy

27/02/2025 2:01 PM



Patient name: Noama Sobhy

Date of examination: 15 May 2025

COLOR DUPLEX EXAMINATION OF THE VENOUS SYSTEM OF LEFT  
UPPER LIMB REVEALED :-

- Patent deep venous system along the axillary and brachial veins limb, with no evidence of venous obstruction.
- Patent superficial venous system (Cephalic and basilic veins).
- Extensive upper limb edema ?? cellulitis for clinical correlation.

Conclusion :

- Patent deep and superficial venous system.

Much obliged,  
Mona Abo Elenin M.Sc.